PRODUCT WRITE-UP

PLAN NAME : IL LADYCARE ADVANTAGE RIDER
PLAN CODE : U59
INITIAL LAUNCH DATE : 31 October 2006
REVISION DATE : 02 May 2012
QUICK REFERENCE : Overview of revised features as follows with effective from 02 May 2012, which are applicable to both existing and new business policies:

- increase the limit of total SA for combination of all the lady riders (ie, GreatLady Riders A, B and C (U27 - U29), IL LadyCare Rider (U58) and IL LadyCare Advantage Rider (U59)) from RM150,000 to RM200,000 **per life**.

PLAN DESCRIPTIONS

1. This is an optional yearly renewable unit deduction rider that can be attached to regular premium investment-linked insurance policy at any time, effectively at next monthly due. The plans refer to:
   - SmartProtect Essential Insurance 2 (Plan Code: 0281)
   - *Great ProtectLink Insurance (Plan Code: 0231/231Y)
   - *GreatLife International Portfolio Insurance (Plan Code: 0266)
   - *Centennial AddVantage Insurance (Plan Code: 0271)
   - *AddVantage Plus Insurance (Plan Code: 0275)
   - *SmartProtect Essential Insurance (Plan Code: 0276)

*Withdrawn Products (for inclusion of rider during CS stage)

2. Insurance charge will be deducted from the unit fund(s) monthly.

3. This rider is made available since 31 October 2006. The re-pricing of this rider will be applied retrospectively, effective 1 June 2011.

PLAN BENEFITS

1. Lifetime Female Benefits

Table 1

Event	Percentage of Sum Assured	Waiting period	Survival Period
Carcinoma-in-situ (Breast, Cervix Uteri, Uterus, Placenta, Fallopian tube, Ovary, Vulva and Vagina)	30%	120 days	30 days
Female Cancers (Breast, Cervix Uteri, Uterus, Placenta, Fallopian tube, Ovary, Vulva and Vagina)	100%	60 days	30 days
Systemic Lupus Erythematosus (SLE) with Lupus Nephritis	100%	30 days	30 days
Facial Reconstructive Surgery due to Accidents (above the neck)	30%	0 day	0 day

Skin Grafting necessitated by accidental burns or skin cancer	30%	60 days (only for skin grafting due to malignant skin cancer)	0 day

Upon diagnosis of any of the specific conditions, a percentage of the sum assured (Table 1) will be payable in one lump sum. Respective waiting period and survival period as stated in the table above are applicable under these benefits. No payment will be made for re-constructive surgery / skin grafting which is not deemed as Medically Necessary. The company will only pay for each diagnosis as stated in the table above **once per policy**. The maximum aggregate benefit payable under this Table is 100%.

2. Female Surgical Procedures

Table 2

Event	**Procedures**	**Percentage of Sum Assured**	**Waiting Period**	**Survival Period**
Breast Related	Breast Lumpectomy - Unilateral	10%	60 days	N/A
	Breast Lumpectomy - Bilateral	15%		
	Mastectomy	25%		
	Breast Reconstructive Surgery	30%		
Female Internal Organ	Total Hysterectomy, Radical Hysterectomy, Radical Vulvectomy, Total Pelvic Exenteration	30%		

A percentage of the sum assured will be payable in one lump sum for any procedures stated in Table 2. All female surgical procedures above must be performed to treat Female Malignant Cancers in order to qualify for a claim. Under the event of Breast related surgical procedures, if there is any earlier claim made, the company will pay the difference of each surgery up to the maximum allowed within any Breast Related category. The formula is as follow:

Minimum (% of sum assured that corresponds to the claimable event, 30% of SA) less any earlier claim(s) made.

No payment will be made for Female Surgical Procedure, which is not deemed as Medically Necessary. The company will only pay for each diagnosis as stated in the table above **once per policy**.

3. Pregnancy & Neonatal Coverage

A lump sum payment of RM 20,000 will be payable if the Life Assured is diagnosed as suffering from any one of the following complications/conditions arising from her pregnancy

Event	Waiting Period	Survival Period
Eclampsia	12 months	30 days (Except stillbirth and neonatal death after childbirth)
Ectopic Pregnancy		
Molar Pregnancy		
Miscarriage due to Accident (20-28 weeks pregnancy)		
Intrauterine Death of Fetus (Stillbirth – 29 weeks pregnancy onwards)		
Disseminated Intravascular Coagulation (after 7 months of Pregnancy)		
Neonatal Death after Childbirth (within 28 days of delivery)		

No payment shall be made for any diagnosis of any specified conditions stated in the table above within the waiting period of 12 months. This benefit will be terminated once a claim is made under this section.This coverage shall automatically be ceased upon the policy anniversary on which the Life Assured attained age 45 next birthday.

4. Infant Coverage

A lump sum payment of RM 20,000 will be payable if the Life Assured's child (up to age 4) is diagnosed to be suffering from any of the congenital conditions:

Event	Waiting Period	Survival Period
Down's Syndrome	12 months	N/A
Spina Bifida		
Atrial Septal Defect		
Ventricular Septal Defect		
Tetralogy of Fallot		
Patent Ductus Arteriosus		
Premature Birth Requiring Neonatal ICU (subject to condition when it is above 30 days)		
Cleft Palate		

This coverage shall commence from the 29th day after the birth of child (except for Premature Birth Requiring Neonatal ICU). This benefit will be terminated once a claim is made under this section.
No payment under this section shall be made if the Childbirth results from fertility treatment, including in-vitro fertilizations.This coverage shall automatically be ceased upon the policy anniversary on which the Life Assured attained age 45 next birthday.

Notes:

a. The rider will be terminated once the Total Claim payout reaches 100% in Table 1 (for Lifetime Female Benefit) **AND** 30% in Table 2 (for Female Surgical Procedures) **AND** one claim under Pregnancy & Neonatal Death **AND** one claim under Infant Coverage.

b. Rider will still be in force and insurance charges will be deducted as long as the claims scenario does not meet Note (a) above.

c. The benefits offered under this rider will only be paid for the first ever occurrence of any covered event that happens to the Life Assured.

DESCRIPTION / DEFINITION OF BENEFITS

1. Lifetime Female Benefits

I. Carcinoma-in-situ

For the purpose of this policy, Carcinoma-in-situ covers only the following sites: Breast, Cervix Uteri, Uterus, Placenta, Ovary, Fallopian Tube, Vagina and Vulva. Carcinoma-in-situ is defined as a focal autonomous new growth of carcinomatous cells, which has not yet resulted in the invasion of normal tissues. Invasion means an infiltration and/or active destruction of tissue or surrounding tissue beyond the basement membrane. The diagnosis of carcinoma-in-situ must always be positively diagnosed based upon a microscopic examination of fixed tissue and supported by a biopsy result. Clinical diagnosis does not meet this standard. No benefit will be payable:-

- for Cervical Intraepithelial Neoplasia (CIN) classification CIN I, CIN II and CIN III (severe dysplasia without Carcinoma-in-situ); and
- if the Life Assured is diagnosed to be HIV-positive.

II. Female Cancers

Focal autonomous uncontrolled growth and spread of malignant cells, which has resulted in the invasion and destruction of normal tissues. This benefit cover primary "Cancer" as follow: Breast, Cervix Uteri, Uterus, Placenta, Ovary, Fallopian Tube, Vagina and Vulva.
The following are excluded:

- Cancers which have metastasized to and do not originate from the above sites;
- Tumours showing the malignant changes of carcinoma-in-situ and tumours which are histologically described as pre-malignant or non-invasive, including but not limited to: Carcinoma-in-situ of the Breasts, Cervical Dysplasia CIN I, CIN II and CIN III;
- All tumours in the presence of HIV infection;

III. Systemic Lupus Erythematosus (SLE) with Lupus Nephritis

A multisystem, multifactorial, autoimmune disorder which affects mostly females in their childbearing years and is characterized by the development of auto-antibodies, directed against various self-antigens. For the purpose of this policy, SLE will be restricted to those forms of systemic lupus erythematosus which involve the kidneys (Type III to Type IV Lupus Nephritis, established by renal biopsy). Other forms, discoid lupus, and those forms with only haematological and joint involvement will be specifically excluded.

World Health Organization ("WHO") Lupus Classification:
Class I (minimal change) – Negative, normal urine
Class II (Mesangial) – Moderate proteinuria, active sediment
Class III (Focal Segmental) – Proteinuria, active sediment
Class IV (Diffuse) – Acute nephritis with active sediment and/or nephritic syndrome
Class V (Membranous) – Nephrotic Sydrome or severe proteinuria.

IV. Facial Reconstructive Surgery due to Accidents

The actual undergoing of re-constructive surgery above the neck (restoration or re-constructive of the shape of and appearance of facial structures which are defective, missing or damaged or misshapen) performed under general aneasthesia by a registered surgeon to correct disfigurement as a direct result of Accident or assault. The surgery must be Medically Necessary and the Accident causing disfigurement must have resulted in the in-patient hospitalization and treatment of the Life Assured.The accident must be caused solely and directly by violent, external and visible means; and the sole, primary and independent cause of the injury (or injuries) requiring reconstructive surgery.

V. Skin Grafting necessitated by Accidental Burns or Skin Cancer

The actual undergoing of:

- skin transplantation due to accidental full-thickness (3rd degree) burns to the face; or
- skin transplantation due to the removal of Malignant Skin Cancer first diagnosed after the waiting period of 60 days.

Malignant Skin Cancer means the uncontrolled growth and spread of malignant cells in the skin and the invasion and destruction of normal tissue. The cancer must be confirmed by histological evidence of malignancy by Consultant Pathologist. Metastatic skin cancer is included. The following are excluded:

- Carcinoma-in-situ
- Skin cancers less than 1.5 mm Breslow thickness or less than Clark level 3 depth of invasion
- Cancers manifesting themselves as complications of AIDS.

2. Female Surgical Procedures

Specified female surgical procedures are defined below:

i. Breast Lumpectomy – Unilateral

The removal of a malignant breast tumour and surrounding breast tissue in one breast without removal of muscles, skin or lymph nodes.

ii. Breast Lumpectomy – Bilateral

The removal of a malignant breast tumour and surrounding breast tissue in both breasts without removal of muscles, skin or lymph nodes.

iii. Mastectomy

The surgical removal of at least one quadrant of the tissue of a breast due to female cancer.

iv. Breast Reconstructive Surgery

The actual undergoing of plastic or reconstructive surgery of the breast following surgical removal of at least one quadrant of the tissue of the breast (Mastectomy) due to female cancer. The surgical removal of breast tissue must be deemed medically necessary by medical adviser. Breast reconstructive surgery must be performed within 2 years after Mastectomy to fulfill the claim under this section.

v. Total Hysterectomy

The removal of the entire uterus due to female cancer.

vi. Radical Hysterectomy (Wertheim's Hysterectomy)

The surgical removal of the uterus, fallopian tubes, upper third of the vagina and pelvic lymph due to female cancer.

vii. Radical Vulvectomy

The surgical removal of the labia majora, labia minora, clitoris and regional lymph nodes due to female cancer.

viii. Total Pelvic Exenteration

The surgical removal of the bladder, lower ureters, pelvic and lower sigmoid colon, uterus, fallopian tubes, vagina and pelvic lymph nodes due to female cancer.

3. Pregnancy & Neonatal Coverage

I. Eclampsia

Maternal hypertension, proteinuria, excessive weight gain, peripheral oedema and coagulation abnormalities where convulsions have occurred and at least two of the following criteria are met:

- Blood pressure 160/100 mm Hg or greater
- Proteinuria >= 3 gm in 24 hour urine collection
- Creatinine > 1.2 mg%
- Oliguria (<500 cc in 24 hour collection)
- Cerebral or visual disturbances
- Pulmonary oedema
- Jaundice
- Intrauterine death
- Thrombocytopenia, coagulopathy
- HELLP syndrome (haemolytic anaemia/microangiopathic anaemia, elevated liver enzymes, low platelets)

A specialist obstetrician must confirm diagnosis.

Pre-eclampsia is specifically excluded.

II. Ectopic Pregnancy

Pregnancy in which implantation of a fertilized ovum occurs outside the uterine cavity (ovary, fallopian tube, abdominal cavity). The Ectopic pregnancy must be terminated by laparotomy or laparoscopic surgery. The diagnosis of Ectopic Pregnancy must be confirmed by an obstetrician or gynecologist.

III. Molar Pregnancy

The end stage of a degenerating pregnancy in which the chorionic villi have formed vesicles and resemble a bunch of grapes. The defining histological criterion of trophoblastic hyperplasia must be present and proven.

IV. Intrauterine Death of Fetus (Stillbirth)

Fetal Death prior to the complete expulsion or extraction from its mother of a product of conception of 28 or more completed weeks of gestation. The death is indicated by the fact that after such separation the fetus does not breathe or show any other evidence of life, such as beating of the heart, pulsation of the umbilical cord or definite movement of voluntary muscles. Elective termination of pregnancy and Abortion are specifically excluded, unless deemed medically necessary by an obstetrician or gynecologist. Abortion refers to the deliberate termination of pregnancy via medical intervention.

V. Miscarriage due to Accident

The termination of pregnancy after commencement of the 20th weeks and prior to commencement of the 29th week of pregnancy, arising from bodily injury caused by an accident to the Life Assured. Following expulsion or extraction from its mother, fetus must at no time have breathed or shown any sign of life. Where the pregnancy is terminated by extraction of the fetus, such extraction must be certified by an appropriate medical acceptance to the Company as being medically necessary. Elective termination of pregnancy and Abortion are specifically excluded, unless deemed medically necessary

by an appropriate specialist. Abortion refers to the deliberate termination of pregnancy via medical intervention.

VI. Disseminated Intravascular Coagulation (After 7 months of Pregnancy)
A life threatening complication of pregnancy, consisting of a systemic thrombo-hemorrhagic disorder, that is characterized by generalized bleeding and end organ damage. The diagnosis must be confirmed by a gynaecologist or obstetrician as disseminated intravascular coagulation and supported by laboratory tests showing a combination of significant thrombocytopenia, pro-coagulant activation, fibrinolytic activation and inhibitor consumption. Disseminated Intravascular Coagulation arising during the first seven (7) months of pregnancy is excluded.

VII. Neonatal Death after Childbirth
The event of death of the Life Assured's Child, within 28 days of delivery, who has followed complete expulsion or extraction from its mother, breathes or shows any other evidence of life, such as beating of the heart, pulsation of the umbilical cord or definite movement of voluntary muscles.

4. Infant Coverage

I. Down's Syndrome
A specific chromosomal abnormality specifically an autosomal aberration, identified by an extra chromosome 21 and characterized by muscular hypotonicity, microcephaly, brachycephaly, and a flattened occiput. A Consultant Pediatrician shall base such diagnosis on the accepted currently applicable criteria of Down Syndrome after full examination. Diagnosis must be supported by retardation of physical and mental development. Down's Syndrome is also known as Trisomy 21 or Mongolism.

II. Spina Bifida
Defective closure of the spinal column due to a neural tube deficit with a resultant meningomyelocele or meningocele and associated neurological deficit. Spina Bifida Occulta is specifically excluded and the diagnosis must be confirmed by a medical specialist.

III. Atrial Septal Defect
A hole in the partition (septum) between the left and right atrium (upper chambers) of the heart permitting abnormal circulation between the left and right side of the heart. The diagnosis must be confirmed by a Paediatric Cardiologist and supported by an echocardiogram where the condition warrants surgical closure for the reversal of haemodynamic abnormalities and the prevention of heart failure, paradoxic embolisation or irreversible pulmonary vascular disease.

IV. Ventricular Septal Defect
A hole in the partition (septum) between the left and right ventricle (lower chambers) of the heart permitting abnormal circulation between the left and right side of the heart. Claims shall only be admitted if the condition warrants surgical closure for the reversal of haemodynamic abnormalities and the prevention of heart failure, paradoxic embolisation or irreversible pulmonary vascular disease, as recommended by a Paediatric Cardiologist and supported by an echocardiogram.

V. Tetralogy of Fallot
An anatomic abnormality with severe or total right ventricular outflow tract obstruction and a ventricular septal defect allowing right ventricular deoxygenated blood to bypass the pulmonary artery and enter the aorta directly. The diagnosis must be confirmed by an

appropriate medical specialist and supported by an echocardiogram, and invasive surgery must be performed to correct the condition.

VI. Patent Ductus Arteriosus
The failure of the channel between the pulmonary artery and aorta to close after birth, where in the opinion of an appropriate medical specialist, the Patent Ductus Arteriosus is unlikely to close spontaneously. The diagnosis must be confirmed by a cardiologist and supported by an echocardiogram, and invasive surgery must be performed to correct the condition.

VII. Premature Birth Requiring Neonatal ICU
A live birth with a gestation period shorter than 37 weeks and birth-weight less than 1800 grams requiring medically necessary confinement in a recognized Neonatal Intensive Care Unit (NICU) of a hospital recognized by the Malaysia Ministry of Health for a period more than 30 days.

VIII. Cleft Palate
A congenital fissure of the palate often associated with the cleft lip. Cases with cleft lip alone will be specifically excluded.

INSURANCE CHARGES

This is yearly renewable monthly deduction rider, insurance charge will be deducted from unit fund throughout the coverage period based on attained age. Insurance charge is not guaranteed.

UNDERWRITING

1. Sum Assured
Fixed Sum Assured: RM50,000

Note:
a) This rider can not be attached to Basic plan with Sum Assured less than RM50,000
b) Any subsequent policy alteration shall be subject to (a) and
c) If IL LadyCare Rider and IL LadyCare Advantage rider are attached to the same basic ILP, the total sum assured of these two rider sum assured can not more than 1 x BSA.

2. Age at Entry
Minimum : 17 years next birthday
Maximum : 40 years next birthday
However, this rider may be renewed up to the Life Assured's Age 74 years next birthday.

3. Policy Term

75 years age next birthday – entry age next birthday

4. Underwriting Requirement
a) The sum assured for this rider is fixed at RM50,000. However, the calculation for maximum Sum Assured per life which include IL LadyCare Rider, IL LadyCare Advantage Rider and GreatLady Rider Plan B is RM200,000 per life.
b) This rider will not be granted to Life Assured who has existing policy/rider that is providing similar type of pregnancy & infant coverage, eg. GreatLady Plan A and Plan C.
c) Maximum Dread Disease Sum Assured covered per life will exclude this rider.

d) "Supplementary Questionnaire For All Riders Applicable To Ladies Only To Be Completed By Life To Be Assured" is required for every application.
e) "Breast Examination Questionnaire To Be Completed By The Attending Doctor" will be required if the life to be assured is required to go for medical examinations.
f) Sum Assured for this rider will be added to the Basic Sum Assured to determine the medical requirement.
g) Current New Business Medical Requirement table will be adopted.

Note:
If the life to be assured has pre-existing cancer, upon underwriting the coverage of Female Cancer, Carcinoma-in-situ and Female Surgical Procedures should be excluded. Thus, it is not recommended to accept this application.

COMMISSION

Not applicable since this is unit deduction rider. Commission is only payable on the Budgeted Premium.

DISCOUNT

Large Sum Assured Discount / Non-smoker Discount / Other Discounts
Not applicable.

RIDERS AND SUPPLEMENTARY BENEFITS

Not applicable.

WAITING PERIOD

Waiting period is determined from:
i) The date of issue of the Policy and/or this Rider; or
ii) The date of inclusion of this Rider if it has been subsequently included to the Policy; or
iii) The date of any reinstatement of lapse on the Policy and/or this Rider
Whichever is the later.

OTHER FEATURES

1. **Cash Surrender Value / Policy Loan / Automatic Premium Loan / Extended Term Assurance / Paid-up Assurance**
   Not applicable.

2. **Backdating**
   Not allowed.

3. **Free-look Period**
   Policyholder is allowed to cancel this rider within 15 days. Under such circumstance, the Company will reinstate the units which have been deducted for the payment of Insurance Charge after net of medical expenses incurred by the Company (if any) in the issuance of this rider.

4. **Assignment / Third Party Policy / Nomination**
   Follow basic plan.

5. **Reinstatement**
   Follow basic plan.

6. **Replacement of Policies (ROP)**
   Not applicable

OTHER PRIVILEGES

1. **Conversion Privilege**
   Not applicable.

2. **Option to Purchase New Policy**
   Not applicable.

EXCLUSIONS

1) The Company will not pay any benefit under this rider if the relevant condition has existed at any time before or at the date of issue of this Policy and/or this rider or at the date of inclusion of this rider or at the date of any reinstatement.
2) This rider will not cover any condition which is caused directly or indirectly by self-inflicted injuries, while sane or insane.
3) No claim for facial reconstructive surgery and/or skin grafting shall be valid for facial disfigurement resulting from any physical or health impairment or disease or resulting directly from an accident which has existed at the date of issue of this rider.
4) The Company will not pay any benefit under this rider if the relevant condition is caused by
   a. Any violation or attempted violation of the law, or resistance to arrest.
   b. The existence of Acquired Immune Deficiency Syndrome or in the presence of any Human Immuno-deficiency Virus infection.
   c. Alcohol or substance abuse/addiction.
   d. Failure to seek or follow reasonable medical advice.

Note: The exclusions highlighted here may not be exhaustive. For full details of exclusions, please refer to the policy contract.

Investment-Linked LadyCare Advantage Rider Benefit Insurance Charge

Age Next Birthday	Annual Insurance Charge for RM 50,000 Sum Assured
17	113.00
18	113.00
19	116.00
20	131.00
21	156.00
22	190.50
23	228.50

Age Next Birthday	Annual Insurance Charge for RM 50,000 Sum Assured
24	273.50
25	327.50
26	378.50
27	423.50
28	460.50
29	494.50
30	520.00
31	531.50
32	527.00
33	521.50
34	516.00
35	502.50
36	480.50
37	456.50
38	439.50
39	423.00
40	418.00
41*	416.00
42*	421.50
43*	444.50
44*	473.00
45*	391.00
46*	412.50
47*	434.50
48*	441.50
49*	442.50
50*	452.50
51*	461.50
52*	466.50
53*	473.00
54*	476.50
55*	476.50
56*	474.00
57*	473.00
58*	468.00
59*	462.50
60*	460.50
61*	458.00
62*	457.00
63*	455.00
64*	454.00
65*	451.00
66*	447.50
67*	444.50

Age Next Birthday	Annual Insurance Charge for RM 50,000 Sum Assured
68*	442.50
69*	438.00
70*	438.00
71*	436.00
72*	434.00
73*	434.00
74*	433.00

**For renewal only*